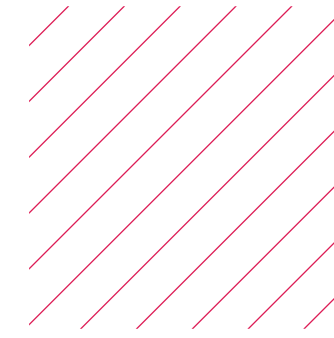




CI INSTITUTE OF NURSING

CERTIFIED NURSE ASSISTANT
TRAINING PROGRAM



Module 15: Observation and Charting

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MODULE 15: OBSERVATION AND CHARTING

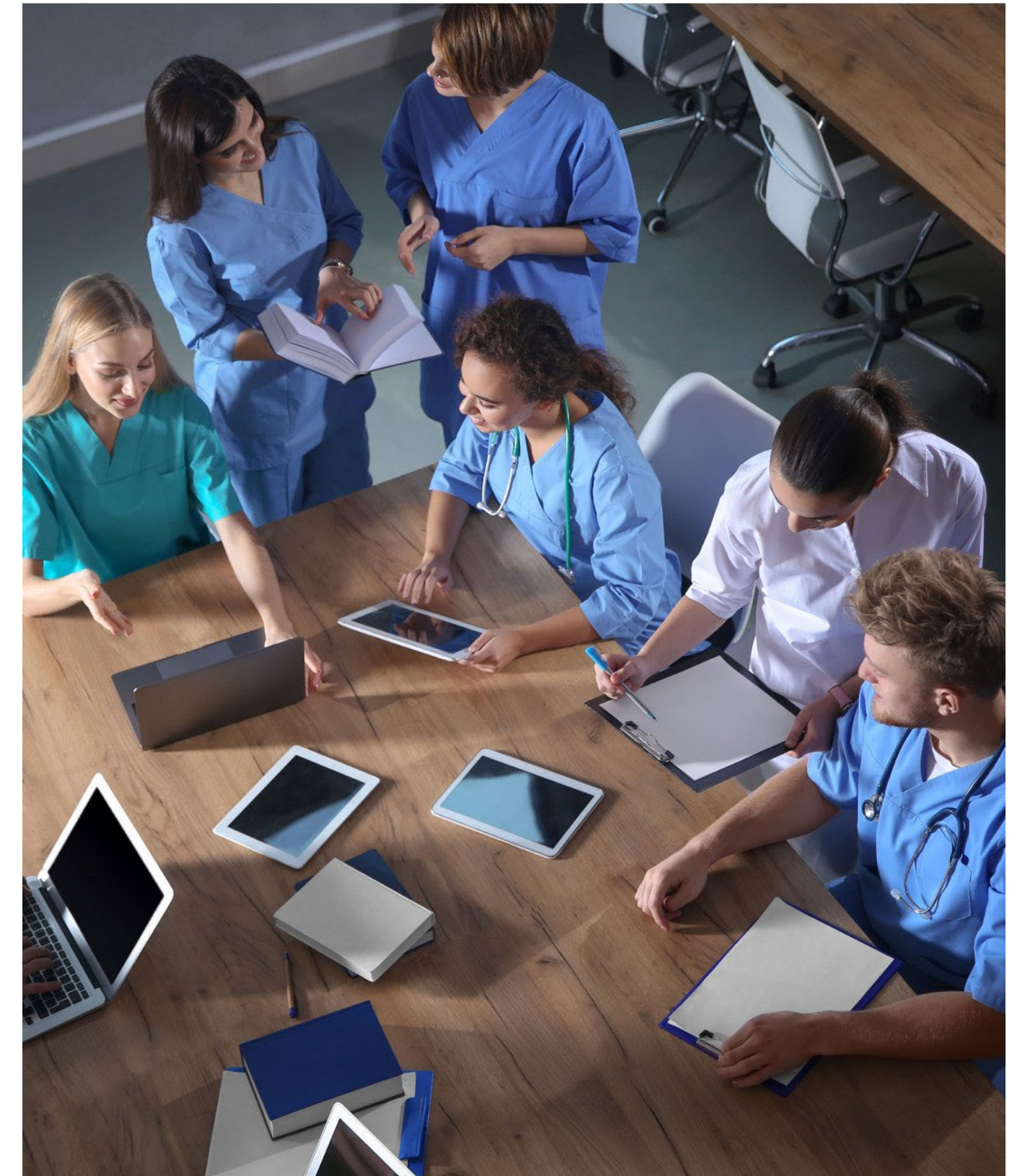
Minimum Number of Theory Hours:

4

Recommended Clinical Hours:

4

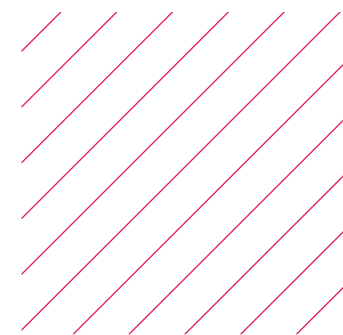
Module 15: Observation and Charting



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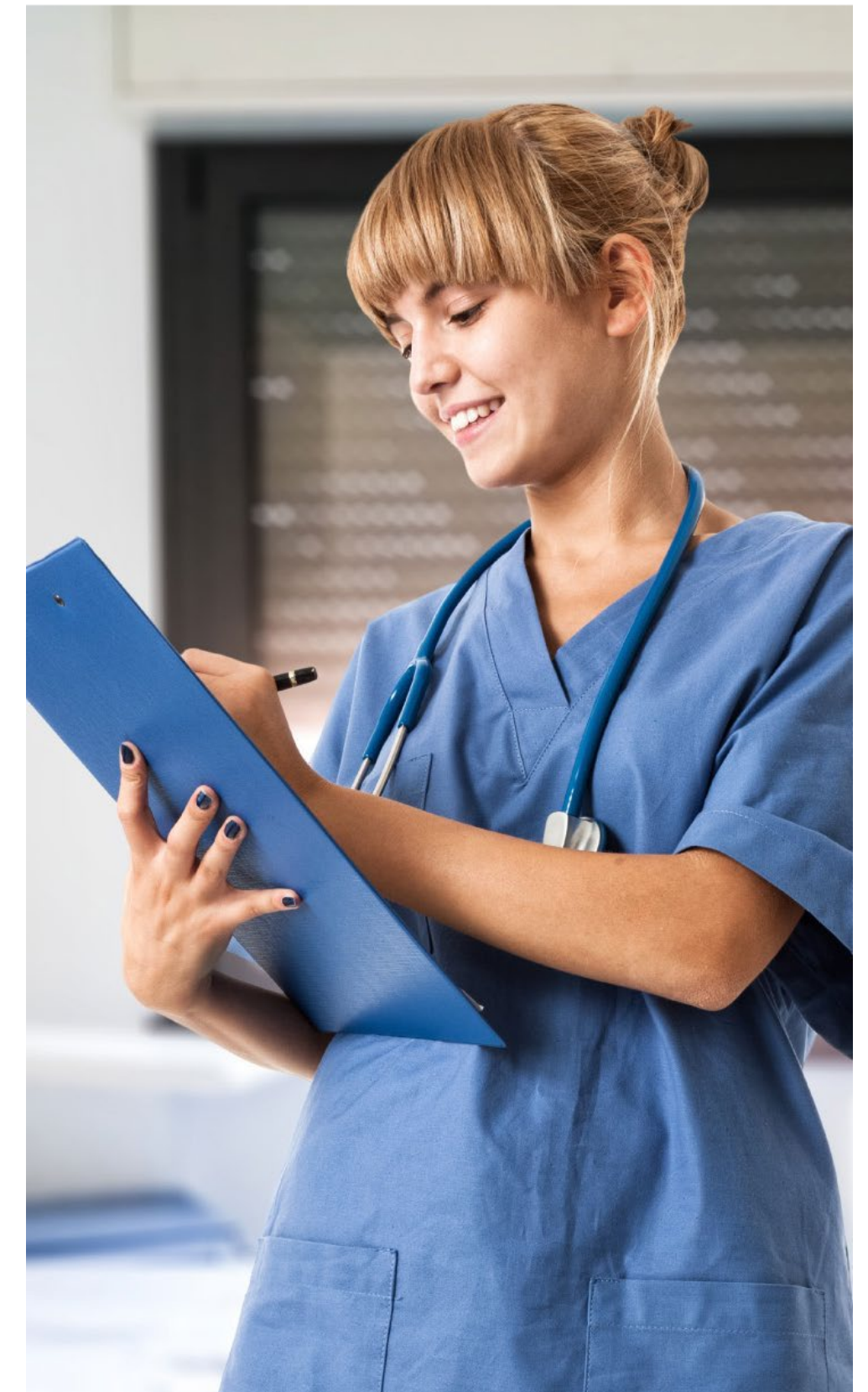
STATEMENT OF PURPOSE

The purpose of this unit is to prepare students to know how, when, and why to use objective and subjective observation skills. They will report and record observations on appropriate documents using medical terms and abbreviations.



TERMINOLOGY:

1. Abbreviation
2. Activities of Daily Living (ADL)
3. Assessment
4. Assessment Reference Day (ARD)
5. Incident report
6. Kardex
7. Minimum Data Set (MDS)
8. Objective
9. Observation
10. Paraprofessional Healthcare Institute (PHI)
<http://phinational.org/about/>
11. Prefix
12. Range of Motion (ROM)
13. Resident Assessment Instrument (RAI)
14. Resident Assessment Protocol (RAP)
15. Patient/resident care plan
16. Root word
17. Subjective
18. Suffix



Patient, resident, and client are synonymous terms referring to the person receiving care

PERFORMANCE STANDARDS

Upon completion of the four (4) hours of class plus homework assignments and four (4) hours of clinical experience, the learner will be able to:

1. Define key terminology
2. Identify word elements used in medical terms
3. Identify medical terminology and abbreviations commonly used in medical facilities
4. Define observation and list the senses used to observe a patient/resident.
5. Describe objective and subjective observations
6. Explain Patient Care Plan and List types of charting documents and the use for each
7. Explain how to accurately complete ADL assessment for MDS
8. Discuss procedures and use of legal issues to use when recording on a patient's/resident's chart



/// DEFINE KEY TERMINOLOGY

- A. Review the terms listed in the terminology section
- B. Spell the listed terms accurately
- C. Pronounce the terms correctly
- D. Use the terms in their proper context



Define observation and list the senses used to observe a patient/resident.

A. Observation – use of senses to collect information about a patient/resident

1. Senses used for evaluation

- a. Sight
- b. Touch
- c. Hearing
- d. Smell

2. Observations should be made with regard to a patient's/resident's

- a. Skin color and temperature
- b. Mood and mental status
- c. Behavior, movement
- d. Unusual odors
- e. Respiration
- f. Ability to respond
- g. Appetite

h. Performance of ADL

i. Elimination

j. Pain or discomfort

Learn to observe the patient/resident throughout daily contacts noting any changes or needs

4. Report all changes or needs to licensed nurse

B. ABCs of observation 1. Appearance

2. Behavior

3. Communication

Describe objective and subjective observations.

A. Objective

1. Signs that you can see, hear, feel and smell
2. Factual, measurable and/or observable signs

B. Subjective

1. What the patient/resident tells you
2. Not directly seen (observed) by the nurse assistant
3. Symptoms reported by patient/resident



Explain Patient Care Plan and List types of charting documents and the use for each

A. Patient/resident record and patient's/resident's chart

1. Communicates and records health history, status, and treatment
2. A legal record

B. Patient care summary/care plan

1. Summarizes physician's orders
2. Identifies critical data such as allergies, code status, diet, activity, etc.
3. Gives medication and treatment information

C. Nursing care plan

1. Lists patient's/resident's need and provides specific nursing activities that address the patient's/resident's needs
2. A guide for the Nurse Assistant for delivering care

D. Graphic sheet - used in some facilities to document

1. Vital signs
2. Intake and output
3. Weight

E. ADL sheet 1. Used to document care at each shift for activities of daily living

2. The record on which most facilities have the care work chart



Explain how to accurately complete ADL assessment for MDS.

A. Minimum Data Set (MDS)

1. Mandated assessment tool by Federal regulation (483.20 (b)) and (F272) to give facility structured, standardized approach to care
2. Basis for payment/or reimbursement to the facility

B. Patient/resident Assessment Instrument (RAI)

1. Each facility must use its state-specified RAI to assess newly admitted patients/residents, conduct an annual reassessment and assess those patients/residents who experience a significant change in status
2. The facility is responsible for addressing all needs and strengths of patients/residents regardless of whether the issue is included in the Minimum Data Set (MDS) or Patient/resident Assessment Protocol (RAPs)
3. The scope of the RAI does not limit the facility's responsibility to assess and address all care needed by the patient/resident

C. MDS SectionG refers to physical functioning and structural problems

1. Bathing
2. Functional limitation in Range of Motion
3. Modes of Transfer

D. Assessment Reference Day (ARD)

1. ARD – set date for the last day of assessment period
2. Assessments are made during the seven days preceding the ARD (the look back period)
3. Example: ARD is Jan. 8. Therefore, the look back period of observation will start from Jan. 2 until Jan. 8



Explain how to accurately complete ADL assessment for MDS.

E. Two categories of ADL section:

1. Patient/resident ADL Self-performance - what the patient/resident actually did for himself or herself and/or how much verbal or physical help was required by staff members during all shifts
2. Patient/resident ADL support provided or needed

F. The reporting of the Nurse Assistant on changes in patient/resident assessment may “trigger” (identify) a problem that needs evaluation and require care planning by the licensed nurse

G. Coding for ADL Self-performance:

1. 0: Independent – no help or staff oversight –OR – staff help oversight provided. Only one or two times during the last seven days
2. 1: Supervision – oversight, encouragement, or cueing provided three or more times during the last seven days – OR – supervision (three or more times) plus physical assistance provided, but only one or two times during the last seven days

3. 2: Limited Assistance – patient/resident is highly involved in activity, received physical help in guided maneuvering of limbs or other non-weight-bearing assistance on three or more occasions – OR

– limited assistance three or more times), plus more weight-bearing support provided, but for only one or two times during the last seven days

4. 3: Extensive Assistance – while the patient/resident performed part of the activity over last seven days, help of following type(s) was provided three or more times:

a. Weight bearing support three or more times

b. Full staff performance of activity (three or more times) during part (but not all) of last seven days

5. 4: Total Dependence – full staff performance of the activity during entire seven-day period. There is complete non-participation by the patient/resident in all aspects of the ADL definition task

6. 8: Activity did not occur – over the last seven days, the ADL activity was not performed by the patient/resident or staff. In other words, the particular activity did not occur at all

Explain how to accurately complete ADL assessment for MDS.

H. ADL support provided:

1. 0 – No set up or physical help from staff
2. 1 – Set up help only
3. 2 – One-person physical assist
4. 3 – Two + persons physical assist
5. 8 – ADL activity itself did not occur during entire seven days